INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Capivasertib + fulvestrant

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
Intervals differ by treatment phase

Regimen source
NCCP 00947
Version 1

Four days on / three off, with fulvestrant injections

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	10
04	Your record and contact card	17
05	Sources and further information	20

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

This combination aims to control advanced breast cancer. Capivasertib blocks a growth signal inside cancer cells and fulvestrant reduces signalling through the oestrogen receptor. Tumour testing helps identify people whose cancer has a relevant pathway alteration.

TARGETED AKT INHIBITOR

Capivasertib

Blocks an internal growth signal called AKT

The tumour has a relevant pathway alteration → Capivasertib blocks AKT signalling → Growth and survival signals are reduced

Blocks AKT1, AKT2 and AKT3, proteins that help carry growth and survival signals inside cells. PIK3CA/AKT1/PTEN alterations can activate this pathway. AKT also helps the body respond to insulin. Blocking it can raise blood sugar, which is why glucose checks start early.

Keep the glucose plan and report diarrhoea, rash or high-glucose symptoms early.

OESTROGEN-RECEPTOR ANTAGONIST AND DEGRADER

Fulvestrant

Blocks and reduces oestrogen receptors

ER-positive cancer uses oestrogen signals → Fulvestrant binds and reduces ER → Less oestrogen-driven signalling reaches the cell

Attaches to oestrogen receptors and reduces their number, making it harder for oestrogen to stimulate hormone-sensitive breast cancer.

Report unusual injection-site bleeding, severe pain, numbness or pain travelling down the leg.

The treatment being discussed

Who this regimen covers

For adults whose ER-positive, HER2-negative advanced breast cancer has a PIK3CA, AKT1 or PTEN change and has grown during or after hormone treatment. The team also checks general health and earlier treatment.

NCCP 00947 v1 · official regimen

Taking the tablets

Take capivasertib with or without food and swallow the tablets whole with water. Do not chew, crush, split or dissolve them. Avoid grapefruit and grapefruit juice.

NCCP 00947 v1 · official regimen

Injection visits

500 mg intramuscularly as two 5 mL injections, one in each buttock, slowly over 1–2 minutes per injection. Cycle 1: days 1 and 15. Cycle 2 onward: day 1 of each 28-day cycle.

NCCP 00947 v1 · official regimen

Ovarian suppression and pregnancy

If you are pre- or perimenopausal, a separately prescribed medicine must suppress ovarian hormone production with this combination. The team may also consider this for men. Age alone does not confirm suppression. Discuss fertility, pregnancy prevention and breastfeeding. Follow the longer fulvestrant contraception advice: two years after its last dose for women; capivasertib advice for men extends sixteen weeks after stopping. Your team confirms the individual plan.

NCCP 00947 v1 · official regimen · EMA Faslodex product information

Why this combination may be chosen

The medicines work on different signals involved in hormone-sensitive cancer. Your team chooses the combination using the cancer tests, earlier treatment and your health, and checks both benefit and side effects as treatment continues.

Discuss your own expected benefit, alternatives and treatment burden with the team. Trial averages cannot predict an individual outcome.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Seek urgent assessment for unusual thirst or urination, severe diarrhoea, inability to drink, or a new rash. Vomiting with abdominal pain, breathing difficulty, fruity breath, confusion or marked sleepiness can signal ketoacidosis: seek emergency help without waiting for a high glucose reading. Blistering or peeling skin also needs urgent help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Planned review points are not equal time intervals or booked tests. Fasting glucose is preferably checked before the dose on day 3 or 4 of a dosing week. The team agrees additional home monitoring.

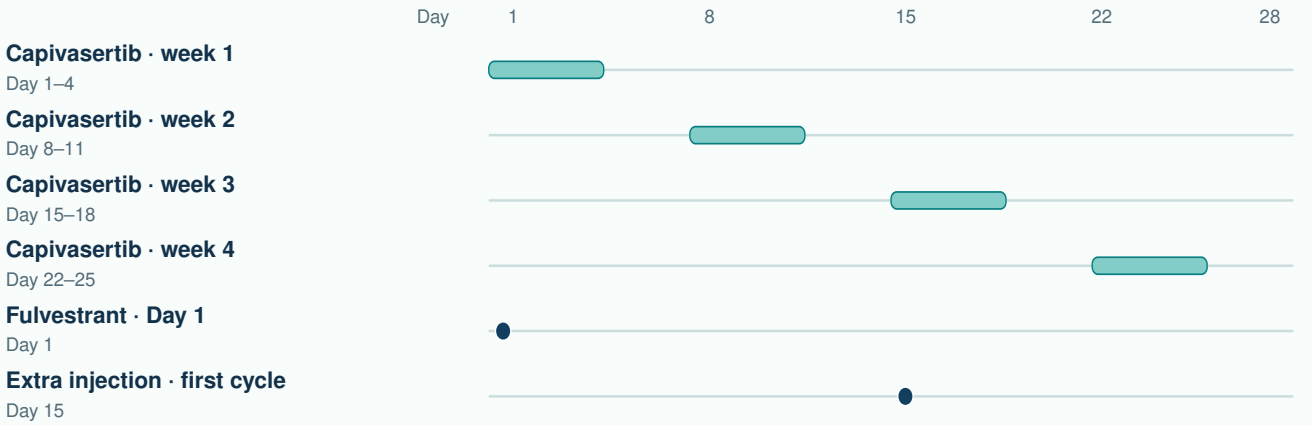
The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00947 v1 · official regimen

THE WHOLE TREATMENT JOURNEY

Capivasertib + fulvestrant: treatment and checks

Cycle 1 · extra fulvestrant loading visit

28-day view



Cycle 2 onward · one fulvestrant visit per cycle

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Gene-altered, ER-positive / HER2-negative advanced breast cancer

Capivasertib is taken twice daily for four days, then stopped for three days, each week. Fulvestrant injections are given on Days 1 and 15 of the first 28-day cycle, then Day 1 of later cycles. Treatment continues while helpful and tolerated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First 2 weeks	Weeks 3–8	After week 8	Pause / changed treatment
Treatment plan NCCP 00947	Confirm biomarkers and prescription	Start four-on/three-off tablets; fulvestrant Day 1	Extra fulvestrant Day 15 in cycle 1; then Day 29	Continue tablet pattern and 28-day injections	Review each medicine before restart
Fasting glucose NCCP 00947	Baseline; optimise control	Weeks 1 and 2; extra checks by risk	Weeks 4, 6 and 8	Monthly; extra checks by risk	Review on and off tablet days if glucose rises
glycated haemoglobin (longer-term blood glucose) (HbA1c) NCCP 00947	Baseline HbA1c	Additional tests if indicated	Week 4 if diabetes, prediabetes or high baseline glucose	Every three months	Individual diabetes review
Blood / kidney / liver NCCP 00947	Baseline full blood count (FBC), renal and liver profile	Early symptoms may need extra tests	Before each 28-day cycle	Before each cycle	Review abnormal results before a restart
Skin / injection checks NCCP 00947	Review skin and bleeding risks	Report rash and injection symptoms	Check skin, hydration and injection tolerance	Continue symptom and medicine review	Treat and follow unresolved effects

What each check is for

1 Confirm the selected pathway

Before starting

For adults whose ER-positive, HER2-negative advanced breast cancer has a PIK3CA, AKT1 or PTEN change and has grown during or after hormone treatment. The team also checks general health and earlier treatment.

Ask / act: Confirm biomarker, endocrine history, ovarian suppression and individual suitability.

2

Glucose and HbA1c

Baseline; weeks 1, 2, 4, 6 and 8; then ongoing

Check blood count, kidney and liver function before treatment and each cycle. Check fasting glucose and HbA1c before starting. Fasting glucose is checked at weeks 1, 2, 4, 6 and 8, then monthly; ideally before the tablet dose on day 3 or 4 of a dosing week. HbA1c is checked every three months, with an extra week-4 check for diabetes, prediabetes or high baseline glucose. Diabetes, new high glucose or metformin initiation needs extra checks under a written plan.

Ask / **act:** Agree who performs, reviews and acts on every result; specify additional home checks.

3

Symptoms and treatment changes

Throughout, including tablet-free days

Seek urgent assessment for unusual thirst or urination, severe diarrhoea, inability to drink, or a new rash. Vomiting with abdominal pain, breathing difficulty, fruity breath, confusion or marked sleepiness can signal ketoacidosis: seek emergency help without waiting for a high glucose reading. Blistering or peeling skin also needs urgent help.

Ask / **act:** Use the urgent contact plan; review the actual capivasertib dose and fulvestrant schedule separately.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Glucose and other checks

Some checks happen between injection visits. Agree who books them and who reviews the results.

Starting and continuing checks

Check blood count, kidney and liver function before treatment and each cycle. Check fasting glucose and HbA1c before starting. Fasting glucose is checked at weeks 1, 2, 4, 6 and 8, then monthly; ideally before the tablet dose on day 3 or 4 of a dosing week. HbA1c is checked every three months, with an extra week-4 check for diabetes, prediabetes or high baseline glucose. Diabetes, new high glucose or metformin initiation needs extra checks under a written plan.

NCCP 00947 v1 · official regimen

If you already have diabetes

Check fasting glucose daily for the first two weeks, then as often as the team advises. An extra HbA1c is recommended at week 4 when diabetes, prediabetes or high baseline glucose is present.

NCCP 00947 v1 · official regimen

If glucose rises

The team may arrange at least twice-weekly checks, including tablet-taking and tablet-free days, and ketone tests or specialist review. Follow the written plan; symptoms can need urgent assessment even without a very high reading.

NCCP 00947 v1 · official regimen

If metformin is started

The team checks kidney function weekly for up to three weeks after metformin starts with capivasertib, then on Day 1 of each cycle. Existing diabetes treatment needs individual reconciliation.

NCCP 00947 v1 · official regimen

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Cycle 1 · extra fulvestrant loading visit

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Capivasertib · week 1	 **Days 1–4** Twice daily
Capivasertib · week 2	 **Days 8–11** Twice daily
Capivasertib · week 3	 **Days 15–18** Twice daily
Capivasertib · week 4	 **Days 22–25** Twice daily
Fulvestrant · Day 1	 **Day 1** intramuscular (IM)
Extra injection · first cycle	 **Day 15** IM
Glucose symptoms / checks	 **Days 1–28** Follow the written plan throughout the week
Diarrhoea / skin symptoms	 **Days 1–28** Report early, including tablet-free days

Weekly tablet breaks: Days 5–7, 12–14, 19–21 and 26–28 have no planned capivasertib. Do not replace these with seven continuous treatment days.

Different medicine schedules: The extra Day 15 fulvestrant dose belongs to cycle 1. Changes to one medicine require review of the other, not an automatic cancellation.

Why these bands stay broad: Tablet days and injection days follow the prescribed regimen. Side effects can occur at any time and are not predicted by the width of a band. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00947 v1 · official regimen ↗

There are no guaranteed "safe days". Tablet-free days still require symptom reporting and the agreed glucose checks. The treatment calendar does not authorise a dose or restart.

Cycle 2 onward · one fulvestrant visit per cycle

Day 1 → Day 28: a general pattern

The same four-days-on/three-days-off capivasertib pattern continues. Fulvestrant is given on Day 1 of each 28-day cycle, without the extra Day 15 loading visit.

Treatment or check	Timing in this view
Capivasertib · week 1	 **Days 1–4** Twice daily
Capivasertib · week 2	 **Days 8–11** Twice daily
Capivasertib · week 3	 **Days 15–18** Twice daily
Capivasertib · week 4	 **Days 22–25** Twice daily
Fulvestrant · Day 1	 **Day 1** IM
Glucose symptoms / checks	 **Days 1–28** Follow the written plan throughout the week
Diarrhoea / skin symptoms	 **Days 1–28** Report early, including tablet-free days

Weekly tablet breaks: Days 5–7, 12–14, 19–21 and 26–28 have no planned capivasertib. Do not replace these with seven continuous treatment days.

Different medicine schedules: The extra Day 15 fulvestrant dose belongs to cycle 1. Changes to one medicine require review of the other, not an automatic cancellation.

Why these bands stay broad: Tablet days and injection days follow the prescribed regimen. Side effects can occur at any time and are not predicted by the width of a band. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00947 v1 · official regimen ↗

Support between treatment visits

Early reporting helps the team treat symptoms and review doses safely.

Diarrhoea plan

Have the agreed anti-diarrhoea plan available before starting. NCCP gives loperamide 4 mg at first onset, then 2 mg after each loose stool, up to 16 mg in a day. Increase oral fluids and contact the team promptly for severe or persistent diarrhoea, inability to drink, dizziness, reduced urine or palpitations. Secondary prevention can be considered for recurrent diarrhoea.

NCCP 00947 v1 · official regimen

Skin plan

Report a new rash promptly. Use moisturisers and any antihistamine or cream prescribed for your symptoms. Do not start steroids or change doses without review.

NCCP 00947 v1 · official regimen

Medicine review

Tell the team about every prescription, non-prescription or herbal product. New medicines can change capivasertib exposure and require a reviewed dose.

NCCP 00947 v1 · official regimen

Cancer response

Your team agrees which scans or other tests you need and when, and reviews symptoms, treatment benefit and your priorities.

NCCP 00947 v1 · official regimen

Practical support

Glucose plan

Agree when to check glucose and when to call. Use your written diabetes-medicine plan on tablet-taking and tablet-free days; do not change existing diabetes medicines yourself.

Diarrhoea and fluids

Have the prescribed diarrhoea medicine available. Follow the agreed early-treatment instructions and contact the team for persistent symptoms, dizziness or reduced urine.

Skin and injections

Report rash early and use skin treatments as advised. Tell the injector about anticoagulants, bruising and any previous leg nerve symptoms.

Taking tablets / attending treatment

Practical treatment details

Make treatment fit into daily life

The usual starting capivasertib dose is 400 mg twice daily, about 12 hours apart, on the prescribed dosing days.

Your prescription may use a lower dose.

Tablets and supply

Capivasertib tablet strengths are 160 mg and 200 mg.

The team confirms the correct number and strength for the prescribed dose.

Access and practical planning

No intravenous (IV) port, infusion or take-home pump is required by this two-medicine regimen.

Additional treatments may have separate arrangements.

Your support contacts

Agree what to do if a test, prescription or injection appointment is missing.

Questions to ask

What if I miss or vomit a capivasertib dose?

If a capivasertib dose is late, it may be taken within 4 hours of its usual time, leaving at least 8 hours before the next dose. If more than 4 hours late, skip it and take the next dose at the usual time. Do not double a dose. If you vomit after a dose, do not take an extra dose; take the next scheduled dose at its usual time. Contact the team if vomiting continues or prevents fluids or medicines.

Do the three tablet-free days also change my diabetes medicines?

Follow the individual written plan from the oncology and diabetes teams. Glucose can fall on tablet-free days. Do not automatically stop or continue a changed diabetes dose without the agreed instructions.

Do I need ovarian suppression?

Pre- or perimenopausal women need a separately prescribed LHRH medicine with this combination. The team may also consider this for men. Age alone does not confirm suppression.

What if the tablets are paused?

Contact the team for a revised plan. Do not restart or increase capivasertib yourself, and do not assume a tablet pause cancels or resets the fulvestrant injection schedule.

How long is treatment and how will benefit be assessed?

There is no fixed number of cycles. The team reviews cancer symptoms, scans, tolerability and priorities, and continues or changes treatment accordingly.

Explain the plan back: Which days have capivasertib, which three days are tablet-free, when are the injections and glucose checks, and which symptoms need urgent help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Capivasertib + fulvestrant · reviewed support plan

Active medicines: Capivasertib + Fulvestrant

Emetogenic risk: Minimal-to-low oral risk

Medicine	Dose / route	When
Loperamide · treatment of diarrhoea	4 mg orally at first onset, then 2 mg after each loose stool; maximum 16 mg in a day	Use at first onset according to the agreed plan; this is not automatic daily prophylaxis Seek assessment for severe/persistent symptoms, dehydration or inability to drink; confirm individual duration and contraindications.

Additional phase-specific support

- None usually required. Do not import cytotoxic antiemetic dexamethasone/aprepitant routines into this regimen.
- Report new rash promptly. Emollients, antihistamines and topical steroids depend on grade and dermatology advice. Early dermatology advice is recommended. Ongoing antihistamines and/or topical steroids may be considered as secondary prevention after persistent or previous grade 3 rash; this is not mandatory routine prophylaxis for everyone.
- Use the individual written glucose and diabetes-medicine plan. The source suggests metformin on capivasertib dosing days and withdrawal when capivasertib is withdrawn unless otherwise clinically indicated. This must be reconciled with pre-existing diabetes treatment: do not tell all patients to stop their usual diabetes medicines on the three capivasertib-free days. Watch for low glucose as well as high glucose on non-dosing days.
- Strong CYP3A4 inhibitors should be avoided where possible; strong/moderate inhibitors require capivasertib dose review. Strong/moderate CYP3A4 inducers should be avoided. Reconcile medicines affecting narrow-therapeutic-index CYP or UGT substrates and sensitive transport substrates. Dose decisions must preserve earlier toxicity reductions.
- Agree glucose equipment, testing windows, result review and urgent contacts before the first dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Capivasertib + fulvestrant

Capivasertib is taken twice daily for four days, then stopped for three days, each week. Fulvestrant injections are given on Days 1 and 15 of the first 28-day cycle, then Day 1 of later cycles. Treatment continues while helpful and tolerated.

Targeted tablets + endocrine injections · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Blood sugar

Capivasertib

New thirst, passing more urine or marked tiredness: contact the team promptly.

3 Skin / mouth

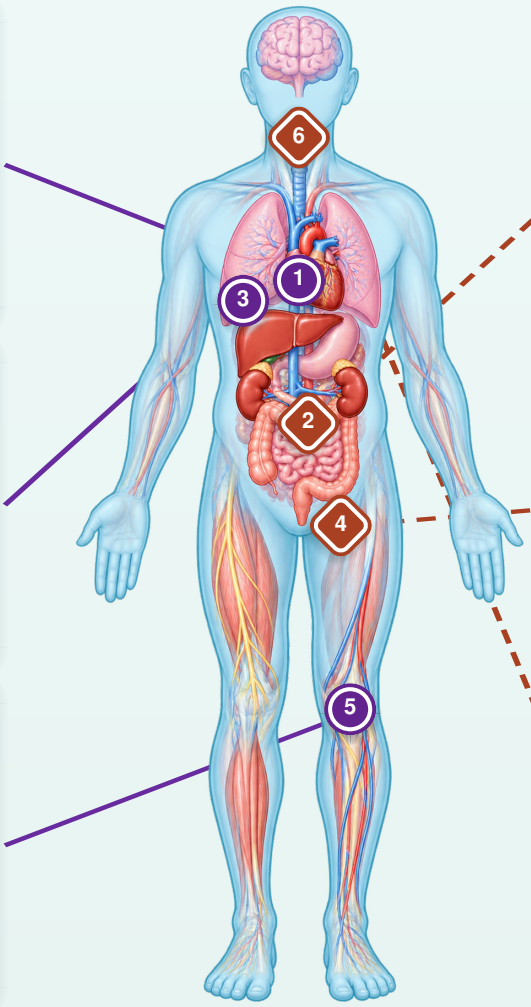
Capivasertib

Report new rash or mouth soreness. Blistering or peeling needs urgent help.

5 Hot flushes / joints

Fulvestrant

Support is available for flushes and joint or muscle symptoms.



2 Bowel / hydration

Capivasertib

Start the agreed diarrhoea plan; seek help if severe, persistent or you cannot drink.

4 Injection sites

Fulvestrant

Report bruising, unusual bleeding, radiating leg pain, numbness or weakness.

6 Jaw / teeth

Capivasertib with bone medicines

Report jaw pain, loose teeth or a wound that does not heal, particularly with bone-modifying treatment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Seek urgent assessment for unusual thirst or urination, severe diarrhoea, inability to drink, or a new rash. Vomiting with abdominal pain, breathing difficulty, fruity breath, confusion or marked sleepiness can signal ketoacidosis: seek emergency help without waiting for a high glucose reading. Blistering or peeling skin also needs urgent help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Thirst / increased urination	☐	☐	☐	
Diarrhoea / dehydration	☐	☐	☐	
Rash / itching / mouth soreness	☐	☐	☐	
Nausea / appetite / fatigue	☐	☐	☐	
Injection pain / bleeding / nerve symptoms	☐	☐	☐	
Hot flushes / joint discomfort	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Seek urgent assessment for unusual thirst or urination, severe diarrhoea, inability to drink, or a new rash. Vomiting with abdominal pain, breathing difficulty, fruity breath, confusion or marked sleepiness can signal ketoacidosis: seek emergency help without waiting for a high glucose reading. Blistering or peeling skin also needs urgent help. Emergency symptoms: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Capivasertib + fulvestrant

NCCP 00947 v1

Capivasertib is taken twice daily for four days, then stopped for three days, each week. Fulvestrant injections are given on Days 1 and 15 of the first 28-day cycle, then Day 1 of later cycles. Treatment continues while helpful and tolerated.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Seek urgent assessment for unusual thirst or urination, severe diarrhoea, inability to drink, or a new rash. Vomiting with abdominal pain, breathing difficulty, fruity breath, confusion or marked sleepiness can signal ketoacidosis: seek emergency help without waiting for a high glucose reading. Blistering or peeling skin also needs urgent help.

https://sactcheck.com/patient/00947/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.20 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00947 v1 · official regimen ↗**
   https://healthservice.hse.ie/documents/10966/00947_Capivasertib_Fulvestrant.pdf
2. **EMA Truqap product information, section 5.1**
   https://www.ema.europa.eu/en/documents/product-information/truqap-epar-product-information_en.pdf
3. **EMA Faslodex product information**
   https://www.ema.europa.eu/en/documents/product-information/faslodex-epar-product-information_en.pdf
4. **NCCP oral antiemetic guidance v6, Table 5**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
5. **Turner NC et al. Capivasertib in Hormone Receptor-Positive Advanced Breast Cancer. NEJM 2023;388:2058–2070.**
   https://www.nejm.org/doi/full/10.1056/NEJMoa2214131
6. **Rugo HS et al. Final overall survival results from phase III CAPItello-291. ESMO Breast 2026, abstract 417O. ESMO Open 11(suppl 4):107440.**
   https://www.esmoopen.com/article/S2059-7029%2826%2901382-7/fulltext
7. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/